

Nutrition Strategies to Reduce Frailty Risk

Good nutrition is one of the most powerful, modifiable tools for preventing and managing frailty. Here's what to prioritise.

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Nutrition plays a central role in both preventing and managing frailty. Unintentional weight loss and inadequate nutrient intake are among the core features used to identify frailty clinically, and malnutrition can both contribute to and worsen the condition. The encouraging news is that nutrition is one of the most modifiable risk factors available, meaning meaningful change is achievable at almost any stage. A registered dietitian can provide personalised guidance, but understanding key nutritional principles can help you make better choices.



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Why Nutrition Matters So Much for Frailty

As we age, nutritional needs actually increase in some ways while appetite and food intake may decrease. This is a dangerous mismatch. The body needs more protein to maintain muscle, more calcium and vitamin D for bones, adequate fibre and fluid for digestive health, yet many older adults consume fewer calories and less of essential nutrients.

Poor nutrition weakens muscle, slows bone healing, impairs immune function, and delays recovery from illness or injury. It's directly involved in the mechanisms that create frailty. Conversely, optimising nutrition can slow frailty's progression and even improve function in people already frail.

Protein: The Building Block of Muscle



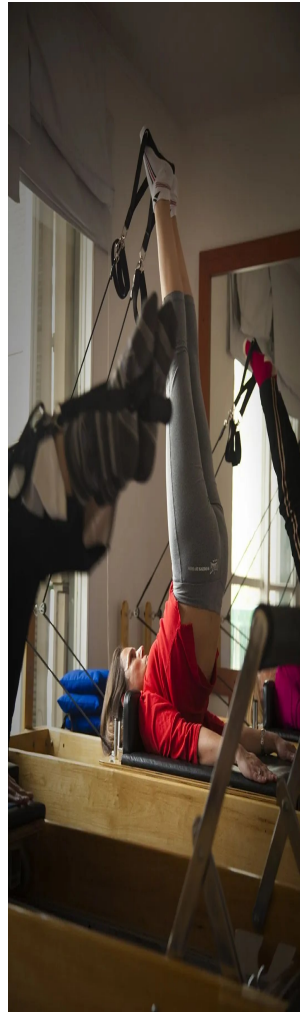
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As we age, the body becomes somewhat less efficient at using dietary protein to build and maintain muscle, a phenomenon sometimes called "anabolic resistance." This means older adults often need more protein, not less, compared to younger adults, to maintain the same muscle mass.

Current recommendations suggest older adults need approximately 1.0-1.2 grams of protein per kilogram of body weight daily. For a 70-kilogram person, that's 70-84 grams daily — roughly 25-30 grams per meal spread across three meals, plus a snack.

Good sources include lean meat, fish, eggs, dairy, legumes, and tofu. The key is including protein at each meal. Some people find concentrated sources like Greek yoghurt, cottage cheese, or a glass of milk at breakfast helpful ways to increase intake without dramatically changing portions.

Key Vitamins and Minerals



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Vitamin D supports both bone and muscle health, and deficiency is common among older adults, particularly those with limited sun exposure. The recommended dietary allowance for adults over 70 is 800 IU daily, but many people need supplementation to achieve adequate levels.

Calcium works alongside vitamin D to protect bone density. Dairy products, fortified plant-based milks, and leafy greens are good sources, though meeting the 1,000-1,200 mg daily recommendation for older adults often requires deliberate effort.

B vitamins support energy metabolism and nerve function. A deficiency in vitamin B12, which is common in older adults and particularly in people taking certain medications for diabetes or acid reflux, can contribute to weakness and cognitive problems.

Adequate fibre and fluid intake support digestive health, which can otherwise affect appetite and nutrient absorption. Older adults sometimes don't drink enough fluid, which can lead to dehydration affecting energy and cognition.

Iron, zinc, and magnesium also play important roles in muscle function and overall health, and deficiencies are more common in older adults than many realise.

Addressing Common Barriers to Eating Well

Several factors can make it harder for older adults to eat enough, including reduced appetite, difficulty chewing or swallowing, limited mobility for shopping and cooking, medication side effects that affect appetite or taste, and social isolation, which can reduce motivation to prepare and enjoy meals.

Dental problems are surprisingly common and often overlooked. Someone with poorly fitting dentures or missing teeth may avoid foods they should be eating. A dentist can help address these issues.

Medications can affect appetite, taste, or nutrient absorption. A pharmacist can sometimes suggest timing adjustments or alternative medications with fewer effects on nutrition.

Limited mobility for shopping and cooking can be addressed through meal delivery services, online shopping with home delivery, or asking family and friends for support. Some communities offer meal delivery programmes for older adults.

Social isolation is both a cause and consequence of poor nutrition. Eating with others — even occasionally — can improve both nutrition and wellbeing. Some communities offer group meal programmes.

Practical Tips for Better Nutrition

Eating smaller, more frequent, nutrient-dense meals can help when appetite is limited. Rather than three large meals, having five or six smaller ones spreads nutrition throughout the day and may feel less overwhelming.

Adding protein or calorie-dense additions like nut butter, cheese, seeds, or olive oil to meals can boost intake without requiring larger portions. A piece of toast with almond butter provides more nutrition than plain toast.

Smoothies made with milk or yoghurt, fruit, and protein powder can be an easy way to increase protein and calorie intake if swallowing is difficult.

Staying hydrated supports overall function and appetite. Many older adults don't drink enough water. Setting reminders or keeping a water bottle visible can help.

For those at higher risk of malnutrition, a registered dietitian can provide a personalised nutrition plan tailored to specific health conditions and preferences. In the UK, a referral to dietetics services can sometimes be obtained through your GP.

Bringing It Together

Combined with regular physical activity, particularly resistance exercise, good nutrition forms one of the two pillars of frailty prevention and management. Small, consistent improvements in diet can have a meaningful impact on strength, energy, and independence over time.

****This article is for general information only and is not a substitute for professional medical advice. Always consult with your healthcare provider or a registered dietitian before making significant changes to your diet, particularly if you have medical conditions or take medications that affect nutrition.****

Q&A

Question: What is the key thing to know about nutrition strategies to reduce frailty risk?

Short answer: Frailty is linked to unintentional weight loss and inadequate nutrient intake. Optimising nutrition — particularly protein intake of 1.0–1.2 grams per kilogram of body weight daily, plus Vitamin D, calcium, B vitamins and adequate fluid — helps manage it. Addressing barriers like reduced appetite or dental issues, and seeking guidance from a registered dietitian, improves outcomes.

Question: When should I seek professional advice?

Short answer: If symptoms persist or worsen, pain is severe and unresponsive to usual relief, or you develop new symptoms, contact your GP, physiotherapist or rheumatology team promptly.

Question: Where can I get free arthritis support in the UK?

Short answer: Arthritis UK: 0800 5200 520. NRAS: 0800 298 7650. Free tools and guides at livingwitharthritis.org.uk.

General information only — not medical advice. Consult your GP or specialist before changing medication or starting a new programme. Emergency: 999. NHS advice: 111. Living With Arthritis · livingwitharthritis.org.uk